



From Freud's dream-work to Bion's work of dreaming: The changing conception of dreaming in psychoanalytic theory

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Bion moved psychoanalytic theory from Freud's theory of dream-work to a concept of dreaming in which dreaming is the central aspect of all emotional functioning. In this paper, I first review historical, theoretical, and clinical aspects of dreaming as seen by Freud and Bion. I then propose two interconnected ideas that I believe reflect Bion's split from Freud regarding the understanding of dreaming. Bion believed that all dreams are psychological works in progress and at one point suggested that all dreams contain elements that are akin to visual hallucinations. I explore and elaborate Bion's ideas that all dreams contain aspects of emotional experience that are too disturbing to be dreamt, and that, in analysis, the patient brings a dream with the hope of receiving the analyst's help in completing the unconscious work that was entirely or partially too disturbing for the patient to dream on his own. Freud views dreams as mental phenomena with which to understand how the mind functions, but believes that dreams are solely the 'guardians of sleep,' and not, in themselves, vehicles for unconscious psychological work and growth until they are interpreted by the analyst. Bion extends Freud's ideas, but also departs from Freud and re-conceives of dreaming as synonymous with unconscious emotional thinking – a process that continues both while we are awake and while we are asleep. From another somewhat puzzling perspective, he views dreams solely as manifestations of what the dreamer is unable to think.

Keywords: Bion, dreaming, dreamwork, Freud

Wir haben die Kunst, damit wir nicht ander Wahrheit zugrunde geben.

[We have art so that we shall not be destroyed by the truth.]

(Nietzsche, in Ondaatje, 1970, Foreword)

Introduction

Dreams paradoxically protect us from and inform us about unknown truths. Dreams paint a timeless montage, brushed freely by remnants of past, present, and possible future experience. The true meaning of a dream can never be known and never told. Dreams are works in progress that – if we are open to them – give us a chance to come to grips with truths that we feel least capable of facing. But those truths are always more than we can bear: in every dream, to some degree, we evade much of what is true to our emotional experience.

For some time now, it has seemed to me that psychoanalysis has moved its focus away from dreams. I think that this, at least in part, reflects

reluctance by practicing psychoanalysts to think about dreams in ways that go beyond Freud's momentous contribution. Simply put, Freud's towering legacy dissuaded subsequent theorists from reconsidering dream-work from new perspectives that require of the contemporary analyst nothing less than a paradigm shift.

Bion makes this shift by putting dreaming at the core of psychological functioning, and moving the emphasis in dream theory "from the symbolic meaning of dreams to the *process of dreaming*" (Ogden, 2007b, p. 577). He is concerned more with the way we dream than with the dream's symbolic content (Bion, 1962a, 1962b).

Dreaming, for Bion, involves the pursuit of truth through thinking and feeling. He believes that the driving force of human development is the search for truth and that the mind is developed through dreaming as we strive to discover what is real about our experience.¹

In this paper, I will develop two of Bion's ideas concerning dreaming: (1) the idea that aspects of all dreams are works in progress that allow some access to veiled truths about ourselves (Bion, 1962a, 1962b); and (2) the idea that all dreams also contain elements that are not works in progress and are the equivalent of visual hallucinations, an idea that Bion proposed to himself (in his *Cogitations* [1992, p. 68]), but never developed or published during his lifetime.

Freud's dream-work and Bion's work of dreaming

Freud rescued the study of dreams from mythmakers, seers, neurologists, and purveyors of the daemonic.² He brought to the understanding of dreams a scientific approach offering new possibilities for exploring and organizing previously unknown and unknowable depths of the unconscious.

Freud (1900a) considered *The Interpretation of Dreams* to be "... the most valuable of all the discoveries it has been my good fortune to make. Insight such as this falls to one's lot but once in a lifetime" (Freud, 1931, p. xxxii). But when Freud (1923) introduced his structural model in *The Ego and the Id*, many felt that it was "... so unrelated to dream analysis as to have appreciably diminished the importance of clinical emphasis on the dream" (Waldhorn, 1967, p. 52). The Kris Study Group suggested that Freud led psychoanalysts away from a focus on dreams by shifting emphasis from the topographic model to the structural model.³

¹While he does not put it in these terms, Bion does say that dreaming is a form of thinking, the most profound form of thinking (Bion, 1965), and that thinking is the pursuit of truth: "... healthy mental growth seems to depend on truth as the living organism depends on food" (Bion, 1965, p. 38).

²Historically, dreams were considered part of mythology. They were used to foretell events through the dream interpreter and were viewed as "a formidable or hostile manifestation by higher powers, daemonic and divine" (Freud, 1901, p. 633). "[T]here may have been a time when a campaign without dream interpreters seemed as impossible as one without air-reconnaissance seems today. When Alexander the Great started on his conquests, his train included the most famous dream interpreters ..." (Freud, 1916a, pp. 85–6).

³The topographic model is an archaeological metaphor cast in terms of the unconscious, preconscious, and conscious "levels" of the mind, and the structural model is a metaphor of a "group" consisting of the id, ego, and superego (Waldhorn, 1967, p. 52).

Curiously, Freud's contributions were so revolutionary that they had the effect of halting further theorizing about dreaming. Freud regretted that his analytic colleagues did not share his enthusiasm for dreams. Many contemporary analysts would disagree with Freud's view that dreams hold a "special place" (Freud, 1933, p. 7) in the practice of psychoanalysis, offering access to "material otherwise unavailable" (p. 7), and believe that the dream is "simply one of many types of material useful for analytic inquiry" (Waldhorn, 1967, p. 57). Freud (1933) stated in his *New Introductory Lectures*: "The analysts behave as though they had no more to say about dreams, as though there was nothing more to be added to dream theory ..." (1933, p. 8) (i.e. as if Freud had already said it all). Freud's disappointment is evident in this statement, but there is also a strong invitation for psychoanalysts to carry forward his investigation of dreams.

Bion's reconceptualization of the theory of dreaming is perhaps one of the great paradigm shifts in psychoanalytic history. Bion viewed his work on dreaming as "... a legitimate extension of an opening for investigation which Freud made but did not follow up" (Bion, 1992, p. 73). While Freud's dream theory provides a model for conceiving of the way in which derivatives of the unconscious become conscious, Bion's dream theory provides a model for how the mind processes emotional experience, giving it meaning. He construes dreaming as the unconscious processing of emotional experience, which occurs continuously and simultaneously with conscious thinking.

In considering differences between Freud's and Bion's theories of dreaming, confusion often arises because they use similar terms to refer to different phenomena. Bion expanded upon Freud's theory of dream-work "without being restricted ... by an existing penumbra of ideas" (Bion, 1962a, p. 2), but at the same time without entirely setting aside Freud's ideas in favor of his own. He wrote: "The term 'dream' I shall always use for the phenomenon described by Freud under that term" (Bion, 1992, p. 95). And again: "The title 'dreamwork' has already a meaning of great value. I wish to extend some of the ideas already associated with it and to limit others" (Bion, 1992, p. 62). These sentences taken from Bion's personal notebook, *Cogitations* (Bion, 1992) (which he never intended to be published), demonstrate how Bion was able to extend the intended meaning of what we think of as a 'dream' by considering the phenomenon from new perspectives.

Freud's concept of 'day residue' is akin to Bion's concept of beta elements. Freud believes that images experienced during the day find their way into the dream through day residues. These give shape to – that is, provide a representational form for – what is unacceptable to the dreamer. Freud believes day residue includes harmless ideas belonging to the preconscious. Bion believes that both daytime experiences and internal experiences are essential to composing a dream. "In Bion's terms, the day residues are 'beta elements' that are linked with inherent and acquired preconceptions" (Grotstein, 2000, p. 14). The task of dreaming is to work on the actual

events, the ‘facts’ of lived experience, which involves the process of transforming beta elements into alpha elements.⁴

In the following historical, theoretical, and clinical overview, I will focus on Freud’s idea that the analyst – not the patient – begins the work of interpreting the unconscious psychological conflict in dreams, and will demonstrate this by examining his work with the ‘Dental Stimulus Dream’ (Freud, 1900b). I will then discuss Bion’s concept of dreaming as our most fundamental way of processing emotional experience. In this connection, I will present a close reading of a paragraph taken from *Cogitations* (Bion, 1992) in which Bion steps back to question his own theory of dreaming, and, in so doing, leaves open for consideration the possibility that dreaming (from a vantage point different from all of Bion’s other vantage points) is a reflection of emotional work that has come to a halt. Finally, by combining several of Bion’s ways of viewing dreaming, I will propose that all dreams can be viewed as unconscious thinking-in-progress, but contain elements that the dreamer is completely unable to dream: beta elements.

Freud’s dream-work

Most readers today are surprised to realize that, for Freud (1901), dreams serve one purpose and one purpose only: they are the “guardians of sleep” (p. 678) – a purely physiological process. Freud (1916a) states: “... [A] dreamless sleep is the best, the only proper one. There ought to be no mental activity in sleep ...” (p. 89). He reiterated this belief almost a decade later when he wrote: “There is only one useful task, only one function, that can be ascribed to a dream, and that is the guarding of sleep from interruption” (Freud, 1925, p. 127).

Freud is saying that the anxiety associated with the threat of the ‘return of the repressed’ may be so great as to awaken a person from his dreams. The metaphor is a pressure-valve metaphor rather than a metaphor of unconscious transformation that mediates psychological growth (in contrast to Bion’s digestive-system metaphor which I will discuss later).

“The state of sleep does not wish to know anything of the external world; it takes no interest in reality, or only so far as abandoning the state of sleep – waking up – is concerned ... all the essential characteristics of dreams are determined by the conditioning factor of sleep” (Freud, 1917a, p. 234). Dreams, as ‘guardians of sleep,’ serve the psychological function of keeping sleep from being overwhelmed by disturbing thoughts. If repressed infantile wishes exert pressure for access to thinking and are uncensored or poorly disguised, they enter the preconscious in a form that is so disturbing as to wake the person up.

Freud does not believe that, by dreaming, the dreamer does psychological work that helps deal with unsettling emotional experience; rather, he believes that dreaming dissipates the threat of overwhelming anxiety caused by the tension of repressed impulses originating in childhood.

⁴Beta elements are raw sense impressions that are changed into alpha elements (mentalized emotional experience) through the process of alpha function, which is a yet-to-be-discovered set of mental functions (Bion, 1962a, p. 38).

Freud believed that we dream only in sleep when the freedom of visual representation that comes with diminished censorship and a state of disconnection of the mind from voluntary musculature allow derivatives of the repressed to appear in dreams. The dream safeguards sleep by acting as a safety valve, allowing sleep to continue by letting off just enough tension from unconscious impulses. If too much libidinal or aggressive energy is released, the capacity for symbol formation through dreaming is exceeded. When unconscious tension builds to such intensity, it wakes the person up. Pressure is transferred from one part of the mind to the other, but the underlying emotional tension continues to exist unless and until *the analyst* makes an interpretation that helps to resolve the unconscious conflict to which it has given rise. The work being done during dreaming alone is limited to monitoring and regulating the 'return of the repressed.'

There is, in fact, no better analogy for repression, by which something in the mind is at once made inaccessible and preserved, than burial of the sort to which Pompeii fell a victim and from which it could emerge once more through the work of spades.

(Freud, 1907, p. 40)

The repressed stays intact; no growth occurs without an analyst to do the work of interpretation.

Freud also explored the way the preconscious and unconscious aspects of mind communicate with each other during dreaming. He made clear divisions between primary and secondary process thinking, unconscious and conscious states, and the states of being asleep and being awake (see Freud, 1911). During sleep, there is a disconnection from the logic (and musculature) of waking life; the focus of one's attention is turned entirely to managing one's internal life (Freud, 1900a, p. 608).

Dream-work translates the unconscious (latent) content (such as childhood fears, conflicts, and wishes that are 'unacceptable to consciousness') into the manifest content. This is done according to principles of (1) *condensation*: "the first achievement of the dream-work" (Freud, 1916b, p. 171), which collapses several concepts into one for purposes of disguise; (2) *displacement*: "... replacing something by an allusion ... [which is] more remote" (Freud, 1916b, p. 174); and (3) *representation*: "transforming thoughts into visual images" (Freud, 1916b, p. 175). Through these disguising mental operations, forbidden wishes are altered in such a way as to give their unconscious derivatives "free access to the conscious [mind]" (Freud, 1915, p. 149). Dream-work accomplishes censorship, editing and compromising, which allows the person to dissipate the sleep-disturbing tension associated with infantile wishes by 'smuggling' them (past the censor) into the preconscious and conscious mind. There is no psychological work (yielding maturational advance) inherent in the disguising and smuggling function of dream-work.

In Freud's dream interpretation, the analyst – positioned between the conscious and the unconscious – interprets the patient's dream, by reversing the direction of dream-work, beginning with the manifest content and working backward to the latent material (Freud, 1916b). Dream-work is "the work which transforms the latent dream into the manifest one; the work which proceeds in the contrary direction, which endeavors to arrive at the latent

dream from the manifest, is our [the analyst's, not the patient's] work of interpretation ... [which] seeks to undo the dream-work" (Freud, 1916b, p. 170).

The dream-thoughts and dream-content are presented to us like two versions of the same subject matter in two different languages. Or, more properly the dream-content seems like a transcript of the dream-thoughts into another mode of expression, whose characters and syntactic laws it is our [the analyst's] business to discern by comparing the original and the translation. Thoughts are immediately comprehensible as soon as we [as analysts] have learnt them.

(Freud, 1900a, p. 277)

With this statement Freud is clearly emphasizing the analyst's primary role in interpreting the patient's dreams, and de-emphasizing the role of the patient in interpreting or understanding his dreams on his own, which is at odds with the way most analysts currently think about dreaming. Freud made an exception for himself in the interpretation of his own dreams. The analyst can, by linking manifest content to latent content, do something more with the dream than the patient is able to do on his own. The analyst's skill in translating the dream's latent content is described by Freud: "When the patient runs out of associations ... we intervene on our own; we fill in the hints, draw undeniable conclusions, and give explicit utterances to what the patient has only touched on in his associations" (Freud, 1933, p. 12). Once achieved, "interest in the dream ... is for the most part at an end" (Freud, 1933, p. 17); "[like] the solution to a jigsaw puzzle, there is no alternative solution" (Freud, 1933, p.12). This and the following comments reflect the degree to which Freud sees the work of dream interpretation as almost exclusively the domain of the analyst. The analyst "... call[s] up ideas that occur to the dreamer till you have penetrated from the substitute to the genuine thing and, on the ground of your own [the analyst's] knowledge, replacing the symbols by what they mean" (Freud, 1916b, p. 170).

Not only does Freud say that the patient engages in no logical productive thinking in the act of dreaming, he also believes that even the patient's associations to the dream do not involve secondary process thinking:

Everything that appears in dreams as the ostensible activity of the formation of judgment is to be regarded not as intellectual achievement of the dream-work but as belonging to the material of the dream-thought and as having been lifted from them into the manifest content of the dream ... I can even carry this assertion further. Even judgments made after waking up on a dream that has been remembered, and the feelings called up in us by the reproduction of such a dream, form part ... of the latent content of the dream and are to be included in the [analyst's] interpretation.

(Freud, 1900b, p. 445, italics in original)

Here, Freud seems to further separate primary from secondary process thinking by viewing even the associations that the patient gives after waking as part of the latent content of the dream.

One of Freud's most quoted ideas – "The interpretation of dreams is the royal road to a knowledge of the unconscious activities of the mind" (Freud, 1900a, p. 608) – is often misleadingly shortened to: 'Dreams are the royal road to the unconscious.' This shorthand obscures the critical idea that Freud was emphasizing, namely, that *the analyst's interpretation of dreams* (not

dreams themselves or the patient's interpretation of his own dream) offers the possibility for understanding unconscious mental activity, in general, and of the dreamer, in particular. The patient's primary roles in dream analysis are that of free-associating, and later thinking and speaking in secondary process logic, in response to the analyst's interpretation of the dream.

Freud's work with a dream

In *The Interpretation of Dreams*, Freud (1900b) presents the 'Dental Stimulus Dream,' which illustrates the importance that Freud attributed to the analyst's interpretation of dreams, and how all interpretations unveil repressed sexual memories, thoughts, desires, and fears. A dream dreamt by a patient of Otto Rank's colleague provides Freud with an opportunity to demonstrate how his theory of dreaming works.

Freud's dream interpretation (carried out through Rank's colleague) follows the steps of dream formation in reverse, working backward from the manifest content – a visit to the dentist to have a tooth pulled. In his preamble, Freud (1900b) tells the reader that the dream is a typical dream in which a "dental stimulus" (p. 387) is used to represent "sexual repression" (p. 387).

Having thus guided the reader in how to think about the dream, Freud presents the dream of a patient being treated by Rank's colleague:

A short time ago I had a dream that I was at the dentist and he was drilling a back tooth in my lower jaw. He worked on it so long that the tooth became useless. He then seized it with a forceps and pulled it out with an effortless ease that excited my astonishment. He told me not to bother about it, for it was not the tooth that he was really treating, and put it on the table, where the tooth (as it now seemed to me, an upper incisor) fell apart into several layers. I got up from the dentist's chair, went closer to it with a feeling of curiosity, and raised a medical question, which interested me. The dentist explained to me, while he separated out the various portions of the strikingly white tooth and crushed them up (pulverized them) with an instrument, that it was connected with puberty and that it was only before puberty that teeth came out so easily ...

(Freud, 1900b, p. 388)

After reporting the dream, Rank states: "... experiences and thoughts from the previous day provide material for an interpretation of the dream" (Freud, 1900b, p. 389). The patient revealed that recently he had had dental treatment and was having continual pain in a tooth in the lower jaw – where, in reality, a dentist had worked longer than the patient had liked. On the day of the dream he had been to the dentist, who suggested that the pain was probably caused by a tooth that needed to be pulled – a 'wisdom tooth.'

Rank was pleased with his colleague's interpretation, and reported to Freud that: "So much for the interpretation [of masturbatory desires] put forward by my colleague, which is most enlightening, and to which, I think, no objections can be raised. I have nothing to add to it ..." (Freud, 1900b, p. 391). Having 'extracted' the repressed material and having offered it to the patient ('in layers') in the form of an interpretation, the analyst's work with the dream is complete, and we are told that Freud, Rank, Rank's colleague, and the patient are all in agreement.

Freud leaves the reader with his thesis: dreams are a product of disguised, unconscious infantile sexual wishes that require the analyst to translate the manifest content of the dream into the ‘language’ of the latent content. I would add to Freud’s formulations the idea that the latent content is originally presented to the preconscious and conscious mind in the ‘language’ of dream imagery and narrative. So when the analyst translates the manifest content of a dream ‘back’ into the language of the latent content, he is in fact translating the language of latent content into a verbally symbolic form for the first time. This verbally symbolized version of the latent content is then presented as an interpretation to the patient and “... makes the motive force ... nothing other than the masturbatory desires of the pubertal period” (Freud, 1900b, p. 385).

By laying out his step-by-step-method, Freud left future generations of psychoanalysts a powerful model for the interpretation of dreams.

Bion’s re-conception of dreaming

[S]omebody here should, instead of writing a book called *The Interpretation of Dreams*, write a book called *The Interpretation of Facts*, translating them into dream language – not just as a perverse exercise [by the analyst], but in order to get a two-way traffic [on the ‘royal road’].

(Bion, 1980b, p. 31)

In this statement, Bion adds to Freud’s conception of working with dreams (the “interpretation of dreams”) his own way of working with emotional experience (the “interpretation of facts”) – which allows for “two-way traffic” between conscious and unconscious thinking. For Bion, psychoanalysis is, most fundamentally, the work of interpreting facts, raw sensory impressions – the elementary ‘facts’; not the work of interpreting dreams, because dreams are themselves the way we have already interpreted the facts – the way we have successfully thought about our emotional experience. Dreams are the interpretation of the facts of the dreamer’s current emotional reality, the “... experience of external or internal psychic reality” (Bion, 1992, p. 45).

The underlying premise of Bion’s conceptualization of dreaming is that dreaming is the primary psychoanalytic function of the mind (Ogden, 2004), and consequently, when we are unable to dream, we are unable to do essential conscious and unconscious psychological work with our emotional experiences. “Failure to eat, drink, or breathe properly has disastrous consequences for life itself” (Bion, 1962a, p. 42). “Failure to use the emotional experience produces a comparable disaster in the development of the personality; ... [which includes] psychotic deterioration ... as [the] death of the personality” (Bion, 1962a, p. 42). He wrote:

The ‘felt need’ [the patient’s need to render meaningful his emotional experience] is very important; if it is not given due significance and weight, the true disease [psychopathology = inability to dream] of the patient is being neglected, is obscured by the analyst’s insistence on interpretation of the dream [i.e. the analyst, in doing the work of dream interpretation, loses sight of the fact that the patient cannot dream].

(Bion, 1992, p. 184)

For a period of time, Bion used his concept of alpha function as a way to conceptualize dreaming. He describes “alpha function” as an “unknown” (Bion, 1962a, p. 38) yet-to-be-discovered set of mental functions, which “transforms the sense impressions [beta elements] related to an emotional experience into alpha elements” (1962a, p. 17). Only after alpha function has turned beta elements into alpha elements do they congeal into a dream-thought suitable for dreaming. “Alpha function transforms sense impressions into alpha elements, which ... may ... be identical with the visual images ... in dreams ...” (Bion, 1962a, p. 7), and provide the mind material with which to create dream-thoughts (Bion, 1967, p. 115), and give emotional problems a symbolic form in which the person may think/dream them (Bion, 1962a; Grotstein, 2000, 2002, 2007; Meltzer, 1983; Ogden, 2003, 2007b).

Bion used the term ‘dream-thoughts’ to refer to the product of the accretion of alpha elements (Bion, 1962a). The dream-thought is composed of linked elements of experience derived from and representing different facets of an emotional problem that needs to be resolved. When the emotional experience remains in the form of unlinked beta elements, they constitute “undigested facts” that cannot be used in the formation of dream-thoughts (Bion, 1962a, p. 6).

Beta elements “are not amenable for use in constructing dream-thoughts, and consequently, are amenable only for evacuation in projective identification” (Bion, 1962a, p. 6), psychosomatic illness, severe perversions, and so on. Projective identification involves psychic evacuation as well as communication: “The patient, even at the outset of life, has contact with reality sufficient to enable him to act in a way that engenders in the mother feelings that he does not want, or which he wants the mother to have” (Bion, 1962a, p. 31).

Bion concludes that the psychotic’s thought disorder and confusion are related to an inability to dream, and said [in an interpretation he made to a psychotic patient] “... without phantasy or dreams you have not the means with which to think out your problems” (Bion, 1962b, pp. 25–6). Because psychotic patients are unable to discriminate between conscious and unconscious experience or to differentiate an actual event in reality from a hallucination or a dream or a fantasy, their dreams are not forms of genuine thinking and do not contribute to psychological growth.

As alpha-function makes the sense impressions of the emotional experience available for conscious and dream-thought the patient who cannot dream cannot go to sleep and cannot wake up. Hence the peculiar condition seen clinically when the psychotic patient behaves as if he were in precisely this state.

(Bion, 1962a, p. 7)

In other words, the ‘dreams’ of the psychotic are “dreams that are not dreams”: no genuine unconscious psychological work is done, and the patient cannot learn from the experience being hallucinated in sleep. In *On hallucination*, Bion suggests that the “[psychotic’s] dreams showed so many characteristics of the hallucination that ... experiences of hallucination in the consulting room might serve to throw light on the psychotic dream” (1958, p. 78).

Bion (1957) believes that both psychotic and non-psychotic aspects of the personality contribute to all dreams. He identifies ‘parts of’ personalities

that seem to be incapable of true dreaming not only in psychotic and borderline psychotic personalities, but also “in psychotic parts of the [normal] personality” (Bion, 1962a, p. 54). These are the parts of the self (even in the healthiest of people) that are dominated by beta/hallucinatory elements, and which are thus unable to think, learn from experience, or do psychological work (Bion, 1957, p. 47).

Analysts who give patients interpretations of their dreams may be missing the fact that some of the dreams or parts of the dreams are disturbing psychic events occurring during sleep that appear to be dreams but are not dreams and do not “warrant the name dream” (Bion, 1962a; Ogden, 2005). In these dream-like imagistic experiences, neither the patient nor the analyst has genuine associations,⁵ and the experience achieves little or no unconscious psychological work.

An example of a ‘dream that is not a dream’ is the dream of a patient who was a Vietnam veteran who was a ‘shooter’ on a gunboat. He suffered from repetitive nightmares in which he dreamt the same dream with minimal differences in content and no variation in emotion. That he was having the same dream each night reflects the fact that he had not done psychological work the previous night, otherwise he would not need to attempt to dream the dream again.

In the dream, the Viet Cong open fire from the darkness, his rifle jams, and he feels paralyzed with terror. He awakes in a panic. If the patient had allowed himself to genuinely dream the dream, he would, I believe, have run the risk of becoming psychotic. By repeating the old (familiar) images, little or no psychological work was achieved, which, for him at the time, was by far the best of the alternatives. He was trying hallucinatorily to control the scene on the river in all its horror – trying to get rid of a terrifying state. And yet, the images persisted night after night. The psychic reality was so disturbing that it was transformed into something like a visual hallucination. Thinking (dreaming) stopped, and action (hallucination) took over.

For Bion, the development of the ability for unconscious psychological work begins in the mother–infant relationship, when the mother’s mind is able to help transform the infant’s beta elements into a form that the infant’s rudimentary alpha function is able to process. ‘Undreamable dreams’ or undreamable parts of dreams might be thought of as beta elements not yet transformed by alpha function – perhaps a residue of what was undreamable in the mother–infant situation or in later, disturbing or traumatic, emotional experiences. Undreamt dream-thoughts remain as split-off pockets of psychosis (Bion, 1962a). Ogden (2007b) refers to this as “undreamable experience” resulting from any of a number of external traumas or intra-psychic forces that “remain with the individual as undreamt dreams in such forms as psychosomatic illness, split-off psychoses,

⁵When, for our own defensive reasons, we do not want to admit to ourselves that we have no associations to a patient’s dream, we can always come up with something. In writing this paper it would be impossible for me to present a patient’s dream or a ‘dream’ that is not a dream without the reader feeling that the patient and I had simply failed to recognize meaningful, emotion-laden elements. The reader necessarily will have associations as he or she would to a Rorschach card, but that does not mean that the card or the ‘dream’ that is not a dream is communicating something meaningful to him.

'dis-affected' states (McDougall, 1984), pockets of autism (Tustin, 1981), severe perversions (De M'Uzan, 1984), and addictions" (Ogden, 2007, p. 7). In these non-dreams, no psychological work is being done; instead, symptoms are generated that are derived from the foreclosed, non-symbolic experience (de M'Uzan, 1984; Schneider, 1995, 2003a, 2003b, 2007, 2009).

Bion (1962a) considered these non-dreams to be a form of non-thinking involving:

... the use of the visual images of the dream for purposes of control and ejection of unwanted... emotional experience. The visual image of the dream is then felt as a hallucinated – that is to say, artificially produced – container intended to hold in, imprison, inoculate the emotional experience the personality feels too feeble to contain without danger of rupture, and so to serve as a vehicle for the evacuatory process.

(Bion 1992a, p. 67)

Ogden describes this as the contained (the dream-thought) overwhelming and destroying the container (the capacity for dreaming) (Ogden, 2004, p. 4). Dreaming is a containing (i.e. thinking) function; but if dream-thoughts are sufficiently disturbing, they (the contained) can overwhelm the capacity for dreaming (i.e. the container).

Implicit in Bion's work is the idea that beta elements that are not processed, but are split off and evacuated, remain unthought emotional experiences. The emotional problem to be solved remains a stagnant, but troubling, psychological presence – because evacuation is never complete. There is, as if for an instant before evacuation, recognition of the undreamable experience, which leaves a disturbing impression (in the psychotic part of the personality) that presses for continuing the work (in genuine dreaming or analysis). Even in 'healthy dreams,' there are both psychotic and non-psychotic elements: "contact with reality is never entirely lost [evacuated] ... the withdrawal from reality is an illusion, not a fact, and arises from the deployment of projective identification ..." (Bion, 1957, p. 46).

In a significant break from Freud, Bion contends that there is no difference between unconscious processing of experience while we are awake and unconscious processing of experience while we are asleep; we are always dreaming our emotional experience. Bion writes: "Freud (1933, pp. 26–7) says Aristotle states that a dream is the way the mind works in sleep: I say it is [also] the way it works when awake" (Bion, 1992, p. 43).

An emotional experience occurring in sleep ... does not differ from the emotional experience occurring during waking life in that the perceptions of the emotional experience have in both instances to be worked upon by [unconscious] alpha function before they can be used for dream thoughts.

(Bion, 1962a, p. 6)

That is, both waking and sleeping experiences are subjected to the same unconscious thinking process by which psychological work is done. The non-psychotic part of the personality is always making meaning of internal and external experience as it (1) transforms beta into alpha elements, (2) links alpha elements in the formation of dream-thoughts, and (3) thinks one's dream-thoughts in the form of dreaming.

Bion believes that dreaming is thinking about emotional experience and that, in the process of dreaming, conscious lived experience is made available to the unconscious for psychological work. Freud, on the other hand, believes that the unconscious mind is not capable of real thinking (logical, secondary process thinking), and that the function of ‘dream-work’ (Freud, 1900b) is to safeguard sleep by protecting the conscious mind against the return of the repressed. Bion believes that patients who bring a dream to an analytic session have already begun doing psychological work in the very act of dreaming. *Patients bring their dreams to analysts not to have their dreams interpreted, but to continue dreaming with the analyst aspects of the dreams that they have not been able fully to dream on their own.*

Bion felt that Freud’s theory of ‘consciousness’ was incomplete and suggested that “the conscious and unconscious [which are] constantly produced together do function as if they were binocular [and] therefore capable of correlation and self-regard” [i.e. self-awareness] (Bion, 1962, p. 54). In other words, they constitute different vertices from which each can look at, and benefit from, the other. In health, we are able to move freely between the conscious and unconscious mind having a two-way conversation through the semi-permeable “contact barrier” (1962a, p. 17) of dreaming, and able to be conscious of some elements of experience, and unconscious of others, while allowing communication between the two.

Echoing Freud, Bion states that, while we are asleep, we shut off most sense impressions coming from the external world, and turn our full attention inward, and that this makes sleep an ideal time for unconscious thinking of one’s emotional experience (dreaming):

One of the reasons why sleep is essential is to make possible, by suspension of consciousness, the emotional experiences that the personality would not permit itself to have during conscious waking life ... for conversion into alpha-elements and a narrative form ...

(Bion, 1992, p. 150)

I wonder if dreams, that is, the actual emotional experiences are not the emotional experiences I do not have, or cannot allow myself to have, during wakefulness.

(Bion, 1992, p. 149)

Bion’s undeveloped idea of dreams as works in progress

I think it is important to find [‘the answers’] out for yourselves. I try to give you a chance to fill the gap left by me.

(Bion, 1980a, p. 5)

Bion believes that in order to advance psychoanalytic theory one must have the freedom to think based upon one’s unique experience. In accord with this belief, in his writing, Bion intentionally presents ideas without offering the certainty of conclusions, and leaves us with a theory of dreaming that includes an invitation for readers to do something of their own with his ideas

(Schneider, 2005a, 2005b, 2005c). “The reader may find the effort to clarify these [obscure ideas] for himself is rewarding and not simply work that has been forced on him because I have not done it myself” (Bion, 1962a, p. ii).

Much of my thinking about dreams as ‘works in progress’ is derived from thoughts that Bion considered, but never developed. In virtually the entirety of Bion’s published writings, he thought of dreaming as the process by which emotional experience is given meaning. That is to say, dreaming is the process by which problematic raw emotional experience (registered as beta elements) is changed into meaningful emotional experience (alpha elements) with which the dreamer can then do unconscious psychological work. The dream is the representation of an emotional experience in the process of moving toward resolution.

However, in the last paragraph of his 10 August 1959 entry in *Cogitations*, Bion speculates that this conception of dreaming may be *entirely wrong*. Perhaps our dreams are comprised exclusively of emotional experiences that we have been unable to think about unconsciously, and which we are therefore evacuating through projective identification in the form of dreams:

... I suggest what is ordinarily reported as a dream [even in health] should be regarded by us as a sign of indigestion, but not simply physical indigestion. Rather, it should be taken as a symptom of mental indigestion. Or, to phrase it more exactly, as a sign when a patient reports a dream to us and we are satisfied that he means by this what we all ordinarily understand by a dream – that there has been a *failure of dream-work alpha*.⁶ The failure of course may be due to precisely such causes as the use of visual imagery in the service of projective identification which I have just been describing [in psychotic patients], but there are other more common causes of failure of dream-work alpha, and there are also degrees of frequency with which the patient resorts to the use of dream imagery in the service of projective identification. *Investigation of the dream [all dreams] as a symptom of a failure of dream-work alpha means that we have to reconsider the series of hypotheses that I have grouped together under the heading of dream-work alpha.*

(Bion, 1992, p. 68, italics added)

Here, using one of his favorite metaphors for thinking – the digestive system – Bion says that the dream may be a “symptom of mental indigestion” that reveals the fact that some emotional experience has not yet been fully digested. Bion states that, when a patient reports a dream: “it should be taken as a symptom” of “a failure of dream-work alpha.” Adding to his theory that dreaming is unconscious emotional thinking, he suggests that all dreams may, at least in part, reflect an inability to process the emotional experience. Rather than representing healthy emotional processing, the dream brought to the analyst, even by a relatively healthy patient, may in part represent the breakdown of emotional processing resulting in the use of “visual imagery in the service of projective identification,” i.e. a form of visual hallucination. Dreams may reveal stalls and limitations in the unconscious thinking process.

⁶With the concept of ‘dreamwork alpha’ (which is synonymous with what he later calls alpha function), Bion adds dream activity occurring in a waking state to Freud’s dreaming while asleep.

Bion implies that there is no reason to have (make) a dream of the sort that the dreamer may remember on waking if the dreamer has already accomplished in unconscious thinking (while asleep) what he needed to accomplish. From this perspective, it is the unsuccessful dream, the un-thought, undreamt dream-thought that is evacuated into visual dream imagery and then brought to the analyst (in its dead-end state) for help in thinking what has been, to that point, a nonexistent, or, perhaps barely begun, thinking process.

Contrary to the idea he had been working on (and a good deal of what he had previously believed about dreaming), Bion considers here the possibility that all dreams (even the dreams of healthy people) may not reflect unconscious thinking about emotional experience, but may instead represent the failure of unconscious thinking which manifests itself in the form of (visual) hallucinatory or evacuatory processes.

Bion is able to say two contradictory things because he is looking at dreaming from different vertices, where the assumptions of one perspective are not the same as the assumptions of the other. Bion demonstrates the essential process underlying his work from beginning to end – that he (and the rest of us) must be free to think every thought from different vertices, no matter how objectionable, illogical, or contradictory they may seem. As if talking to himself, Bion seems to pose the question: what kind of thinking would dreaming be? (Bion, 1992, p. 67). He tries out the idea that dreaming is generative thinking: “For the true dream *is* felt as life-promoting” (p. 67). But then he seems to be asking himself: what if I think of dreaming as a manifestation of all that cannot be dreamt or thought unconsciously? That would change everything. Bion is trying out the idea that dreams are visual hallucinations. “It [the visual hallucinatory experience in sleep] is suspect as being only the *appearance* of a life-promoting phenomenon and so the patient’s sense of being unable to dream is unaffected” (pp. 66–7), i.e. the patient is correct in his sense of being unable to dream. Bion is suggesting that patients naturally guard against hallucinatory parts of dreams, which require “the mind of another” to be dreamt. Finally, Bion seems to be asking himself (and the reader): what are you going to do with these contradictory thoughts? He asks this question because he has to be able to think every thought – even the thought that negates everything that has led up to it.

Bion’s thinking in this paragraph seems to follow the shape of a dream. His mental meanderings reflect valuable, unconstrained primary process thinking, but, as in a dream, this is interwoven with secondary process thinking. I believe that the paragraph contains a valuable idea that Bion left for others to develop: that all dreams are visual hallucinations in sleep and therefore reflect the unthought, unthinkable residue of unconscious thinking.

Dreams as works in progress and works in stasis

Based on my clinical work with patients’ dreams, it seems to me fruitful to combine Bion’s two ways of viewing dreams that I have just discussed. From that binocular perspective, dreams contain (1) elements on which some psychological work has been done, and (2) elements on which no work has

been accomplished. Even in dreams that yield a great deal of self-understanding, there are undreamt parts.⁷ It is as if in dreams we are in conversation with ourselves, and much is left unsayable and unthinkable. The unthought and unthinkable parts of the dream reflect the fact that we are evading those truths that we most fear.

Much has been said about patients who bring dreams to the analyst composed of emotional experiences that are too disturbing for them to deal with on their own, and how we – the analytic pair – continue dreaming the disturbing emotional experience together. As an analyst, I am, of course, interested in the parts of a patient's dream that the patient has already substantially dreamt, i.e. the parts in which the emotional work has already been done. But, as Bion said: "... what you know, you know – we needn't bother with that. We have to deal with all that we don't know" (Bion, 1987, p. 158). In other words, in my work, I am at least as interested in the undreamt parts of the patient's dreams – the unthinkable, unverbalizable, uninterpretable parts – the analysis of which may extend the patient's self-understanding to include an awareness of the 'unthinkable.' I am very interested in what the patient is trying to dream, but cannot dream – the hallucinatory elements in dreams. Working with the hallucinatory aspect of dreams usually involves attending to my reverie states and other sensory responses that I experience while the patient is telling me his dream. In this way I help the patient continue (or begin) dreaming the undreamt portions of his dream.

Although I find that every dream contains hallucinatory elements, some are more obvious than others, and some are so subtle that I do not easily recognize them, or miss them altogether. Nightmares are a form of incomplete dreaming "disrupted at a point where the individual's capacity for generating dream-thoughts and dreaming ... is overwhelmed ..." (Ogden, 2004, p. 5). In nightmares, patients have done the work up to the point that they become so frightened by the dream-thought that they awaken in a state of fear requiring "the mind of another person – 'one acquainted with the night' – to help [them] dream the yet to be dreamt aspect of [the] nightmare" (Ogden, 2004, p. 5).

Dreams other than nightmares contain clear hallucinatory elements. Hallucinatory elements can be reflected in other ways, for example, as an absence of associations, psychotic non-thinking, or somatic responses on my part or on the patient's part.

I will now present a clinical example involving a dream that is a work in progress, a dream that allowed the patient, Mrs. B, and me to continue the work that had begun in her experience of dreaming, and which involved hallucinatory elements. I use the term *dreaming* in its broadest sense of being able to do unconscious psychological work, which is not necessarily visually representational in form.

Most of my analytic work with Mrs. B focused on her responses to her teenage son's dying in a car accident while he was traveling to compete in a

⁷When I say 'unable to dream,' I am not referring to that ultimate mystery in ourselves which cannot be put into visual images or dream thoughts, which can never be brought into our experience, and always remains unknown ... the mystery that Bion (1970) refers to as 'O' (see Grotstein, 2000).

swim meet. At the outset of this 4-times-weekly analysis, the patient was tormented by self-blame which her former husband cruelly played upon by saying: 'You allowed the boy to drive in the middle of a rain storm.' She felt that her life had ended when her son died, as if she 'had fallen into a deep dark hole and died alongside him.' She developed panic attacks and carried a paper bag into which she breathed during an attack.

In our sessions, she held herself perfectly still [stiff] and tightly composed on the couch. Her few responses to my comments came only after long silences. When she did respond, she barely moved her lips as she spoke. As I watched Mrs. B lying stiffly on the couch, I had the image of a corpse I had seen on display prior to cremation on the banks of the Ganges River. She told me she felt nothing, she had no existence without her son, her future was gone, her life was over.

Mrs. B experienced me as blaming her for her son's death as her husband did – and as she herself did. Although she came to analysis with an emotional problem to be solved, she felt the only solution was 'for you to bring him back from the dead – I want him back.' She had no desire to be alive without him. Very concretely, Mrs. B was coming to me to give her back her son. She did not want to engage in thinking or analysis – what good would that do her in her effort to retrieve her son? I could feel how awful it was for her to have lost her son, and wished I could bring him back to her.

Mrs. B was unable to use my interventions, which led me to doubt myself as an analyst. My interventions contained a defensive, 'overly empathic' 'knowing' and 'understanding.' I was responding in ways that were not only un-analytical, but also mechanical, and served to protect me against my own feelings of deadness and ineffectiveness.

At this point in our work, the few dreams the patient had reported contained undifferentiated figures, in which she changed into her son and back again into herself. In so doing, she magically carried her son back from the dead. She could not dream the death of her son. I entered into that inability to dream – instead, I was slipping into wishes for a magical reincarnation.

After some time my emotional experience during our sessions began to include dissociated sensory and somatic experiences. I found myself rubbing my fingertips because they were cold and tingling, as if going numb. As I looked at my hands, I felt confused. Was I cold? Were my fingertips losing sensation? Were they *my* fingertips? Later, I understood my somatic response as withdrawal into a psychotic state lacking depth of thought, emotion, and the capacity for self-reflection – a thorough preoccupation with numb fingertips that did not feel like my own. Disconnected from my body, I was unable to make analytic use of my bodily sensations.

On another occasion, when I met with Mrs. B in the waiting room, my stomach felt upset and was gurgling (I was not experiencing hunger), and I tightened my abdominal muscles to quiet my stomach. [In retrospect, I think I was trying not to wake her or myself from the deadness she/we continued to feel or, perhaps more accurately, that we continued not to feel.]

At some point during this period of the analysis, I came to view differently my earlier reverie of the Ganges River – and the Hindu (magical) belief that if one is cremated during the correct phase of the lunar cycle,

through incarnation, one will magically come back from the dead – i.e. be reborn. This reverie increasingly felt to me to reflect my own wishful/omnipotent response to the patient's magical wish that I 'bring her son back from the dead.' I also understood my bodily reactions as an attempt to hold and digest for her what was, for her, unholdable and undigestable, and, for the time being, undreamable. With this awareness I was able to release the tension in my abdominal muscles and let go of my 'omnipotent control.' I began to feel something beyond the 'knowing' and 'understanding' responses I had felt earlier. I experienced a new awareness, a bodily sensation of my own that I expressed through my tone of voice. In this way, I was on the threshold of bringing together the sensory and the real – the beta elements and the alpha function – to be used for thinking. I was able to help the patient to begin to dream what had been undreamable – to recognize and acknowledge that her son was separate from her and dead.

Shortly after this experience, in a dream that felt somewhat different from the others, Mrs. B dreamt that she was swimming underwater and only the bubbles from her breathing were surfacing. She followed the bubbles to the surface and took a deep breath. As the dream continued, her son appeared in the center of an otherwise empty stage. She approached him from the audience and felt herself hugging him. "I actually felt his physical presence – could smell his essence and feel the softness of his skin against mine." This physical sensation of holding her son stayed with her when she awoke, and provided her with a way to begin to live with the loss – to feel not only a deep sadness, but also a sense of calm. By making the loss physically real, she was able to complete (as much as possible) putting her son to rest, and 'resurfacing' in her life, reclaiming her own life as separate from her son, but with a place for him. Moving beyond the limitations of magical non-thinking, she achieved the capacity to dream.

In retrospect, my initial response to Mrs. B (i.e. superficially 'understanding' and 'knowing') involved a wish to offer containment, an unconscious desire to magically bring back her son. Rather than dreaming about what she could not dream, I took in what she evacuated into me and let it 'stagnate' further (in the form of my magical thinking). Only when I became aware of my omnipotent wish to bring back her son, and could be aware of my bodily sensations as my own, was I able to move beyond the place where she could dream no further on her own (and, in important ways, was not able to dream at all). What began as a delusion developed into reveries, and into forms of waking dreaming that allowed me to think and dream with her. This was a new creation that was entirely different from hallucinatory experience; what had started as a delusion developed into reveries that allowed me to think and dream with Mrs. B.

Conclusion

Freud believes dreaming does not inform or resolve conflicted unconscious emotional experience for the dreamer on his or her own; rather, dreams are fully interpretable and understandable only by the analyst, who puts the

repressed meaning (latent content) of the dream into words for the patient. The analyst, not the patient, does the work of interpreting the dream by connecting latent content to manifest content, thereby giving the dream verbally symbolized meaning. The patient then works with the analyst's interpretations of the dream.

Dreaming, for Bion, is the primary psychoanalytic function of the mind. It involves the pursuit of truth by means of thinking and feeling; the mind is developed through dreaming as we strive to discover what is true to our experience. Bion's 'work of dreaming' allows conscious lived emotional experience to become available to the unconscious for dreaming, so that a person may better look at different ideas about, or solutions to, an emotional problem. The patient's capacity for alpha function underlies his ability to process raw emotional experience, that is, to make it meaningful and available for unconscious psychological work.

Extending Bion's ideas, I have suggested that dreams are psychological works in progress and yet regularly contain elements that the dreamer has been unable to dream because of the disturbing nature of the emotional experience. Bion believes that there is a psychotic and non-psychotic part of every personality; I am suggesting that there is a psychotic and non-psychotic part of every dream. The non-psychotic part tells us what the patient knows (is able to think or begin to think) while the psychotic part presents to us what the patient is unable to think. The latter is the part of the dream that the patient is unable to dream and constitutes something akin to a visual hallucination in sleep. The analytic work with this part of the dream involves both analyst and patient developing the capacity to dream (that is, to unconsciously process) the previously undreamt part of the dream.

Translations of summary

Zwei Von Freuds Traumarbeit zu Bions Arbeit des Träumens: das Verständnis des Träumens in der psychoanalytischen Theorie und seine Veränderungen. Bion entwickelte die psychoanalytische Theorie weiter, indem er Freuds Theorie der Traumarbeit zu einem Konzept des Träumens ausarbeitete, das dieses als den zentralen Aspekt des gesamten emotionalen Geschehens betrachtet. In diesem Beitrag fasse ich zunächst die historischen, theoretischen und klinischen Aspekte des Träumens in der Sicht von Freud und Bion zusammen. Sodann formuliere ich zwei miteinander zusammenhängende Überlegungen, die meiner Ansicht nach für Bions Abspaltung von Freuds Verständnis des Träumens relevant sind. Bion war überzeugt, dass sämtliche Träume fortlaufende psychische Arbeit seien, und vertrat die Ansicht, dass alle Träume Elemente enthalten, die visuellen Halluzinationen ähneln. Ich erforsche und elaboriere Bions Überlegungen, dass alle Träume Aspekte des emotionalen Erlebens enthalten, die derart verstörend sind, dass sie nicht geträumt werden können, und dass der Patient in der Analyse einen Traum präsentiert, weil er hofft, mit Hilfe des Analytikers die unbewusste Arbeit zum Abschluss bringen zu können, die vollständig oder partiell allzu verstörend für ihn war, um von ihm geträumt werden zu können. Freud betrachtete Träume als psychische Phänomene, die das Verständnis der psychischen Funktionen erleichterten, glaubte aber, dass sie lediglich „Hüter des Schlafs“ seien, also kein Medium unbewusster psychischer Arbeit und Weiterentwicklung, solange sie nicht vom Analytiker gedeutet würden. Bion erweiterte Freuds Überlegungen, wich aber auch davon ab, indem er das Träumen mit unbewusstem emotionalen Denken gleichsetzte – einem Prozess, der sowohl im Wach- als auch im Schlafzustand aktiv ist. Unter etwas anderer Perspektive betrachtete er die Träume lediglich als Manifestationen dessen, was der Träumer nicht denken kann.

Del trabajo del sueño de Freud al trabajo del soñar de Bion: La concepción cambiante del soñar en la teoría psicoanalítica Bion desplazó a la teoría psicoanalítica desde la teoría del trabajo del sueño de Freud, hacia un concepto del soñar como aspecto central de todo funcionamiento emocion-

al. En este artículo el autor revisa primero los aspectos históricos, teóricos y clínicos del soñar según Freud y Bion. Luego propone dos ideas interconectadas que reflejarían el corte de Bion con Freud respecto a la comprensión del soñar. Bion consideraba a todos los sueños procesos psicológicos, y en un momento sugirió que todos contenían elementos semejantes a las alucinaciones visuales. El autor explora y elabora las ideas de Bion acerca de que todos los sueños contienen aspectos de experiencia emocional que son demasiado perturbadores para ser soñados, y de que en el análisis el paciente trae un sueño con la esperanza de recibir la ayuda del analista, a fin de completar el trabajo inconsciente que fue demasiado perturbador, total o parcialmente como para ser soñado por su propia cuenta. Freud considera a los sueños como fenómenos mentales que permiten comprender cómo funciona la mente, pero cree que son solo los 'guardianes del dormir' y no, en sí mismos, vehículos de trabajo psicológico inconsciente y de crecimiento hasta que son interpretados por el analista. Bion amplía las ideas de Freud, pero también deja atrás a Freud y re-concibe el sueño como sinónimo de pensamiento emocional inconsciente, un proceso que continúa tanto cuando se está despierto como cuando se está dormido. Desde otra perspectiva, algo desconcertante, considera a los sueños solamente como manifestaciones de lo que el soñador es incapaz de pensar.

Du travail du rêve de Freud au travail du rêve de Bion: une nouvelle conception du rêve dans la théorie psychanalytique. Bion a transformé la théorie psychanalytique freudienne du travail du rêve en une conception du rêve qui place celui-ci au centre du fonctionnement émotionnel. L'auteur de cet article commence par passer en revue les aspects historiques, théoriques et cliniques du rêve dans l'œuvre respective de Freud et de Bion. Il examine ensuite deux hypothèses, liées entre elles, qui lui semblent illustrer le clivage entre les conceptions bionienne et freudienne du rêve. Bion considérait tous les rêves comme un travail psychologique en cours, suggérant à un moment donné qu'ils renfermaient des éléments apparentés aux hallucinations visuelles. L'auteur explore et développe l'idée de Bion selon laquelle tous les rêves contiennent des aspects de l'expérience émotionnelle dont la turbulence est telle qu'elle dépasse la capacité de rêver. En analyse, le patient amène un rêve dans l'espoir de voir l'analyste l'aider à parachever le travail inconscient qui, du fait de son extrême turbulence partielle ou totale, a excédé les capacités du rêveur. Freud considérait les rêves comme des phénomènes mentaux permettant de comprendre le fonctionnement psychique. Il envisageait les rêves comme des 'gardiens du sommeil' et non comme des vecteurs du travail et du développement psychique inconscients jusqu'à ce qu'ils soient interprétés par l'analyste. Bion a non seulement élargi les idées de Freud, mais il s'en est également écarté en faisant du rêve le synonyme de la vie émotionnelle inconsciente – un processus qui se poursuit que nous soyons réveillés ou endormis. Dans une autre perspective quelque peu déroutante, Bion conçoit les rêves comme des manifestations de ce que le rêveur est incapable de penser.

Dal lavoro onirico di Freud al 'Lavoro del sognare' di Bion: Mutamenti nella concezioni del sogno nella teoria psicoanalitica. Bion ha trasformato la teoria freudiana sul lavoro onirico in una concezione che pone il sogno al centro del funzionamento emotivo. In questo lavoro, passo dapprima in rassegna gli aspetti storici, teorici e clinici, rispettivamente secondo Freud e Bion. Propongo poi due idee collegate fra loro che ritengo riflettono una divergenza da parte di Bion dal pensiero freudiano a proposito della concezione del sogno. Bion riteneva che tutti i sogni costituissero lavori psicologici in corso; ad un certo punto egli propose l'idea che tutti i sogni contengano elementi simili ad allucinazioni visive. Esploro qui ed elaboro le ipotesi bioniane secondo le quali tutti i sogni conterrebbero aspetti della vita emotiva che sono troppo inquietanti per essere sognati; il paziente porterebbe il sogno in analisi nella speranza di ricevere l'aiuto dell'analista a completare il lavoro inconscio, lavoro totalmente o parzialmente troppo inquietante per essere sognato dal paziente in modo autonomo. Freud concepiva il sogno come fenomeno psichico che consentiva la comprensione del funzionamento della mente, ma riteneva che, prima di essere interpretato dall'analista, esso fosse unicamente 'guardiano del sonno' e non, in sé, veicolo per il lavoro psicologico inconscio e occasione di crescita. Bion porta avanti le idee freudiane, ma si distacca al tempo stesso da Freud nella sua nuova concezione del sogno come sinonimo di pensiero emotivo inconscio. Quest'ultimo si verificherebbe sia allo stato di veglia che durante il sonno. Da un'altra prospettiva che potremmo in un certo qual modo definire sconcertante, Bion considera i sogni unicamente come manifestazioni di ciò che il sognatore è incapace di pensare.

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